

Patient ID: Parker

Period: May 4 to June 1, 2026

Report Date: June 2, 2026

Coverage: 655/658h (100%)

Patient's monitoring period was less than 30 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 29 days from May 04 to Jun 01

■ HR ■ Breathing



Last 24 Hours

Vital	Avg	Min	Max
Heart Rate	62 bpm	42 bpm	76 bpm
Breathing	17 brpm	11 brpm	24 brpm

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
2	2h	Low Heart Rate	<1	100%

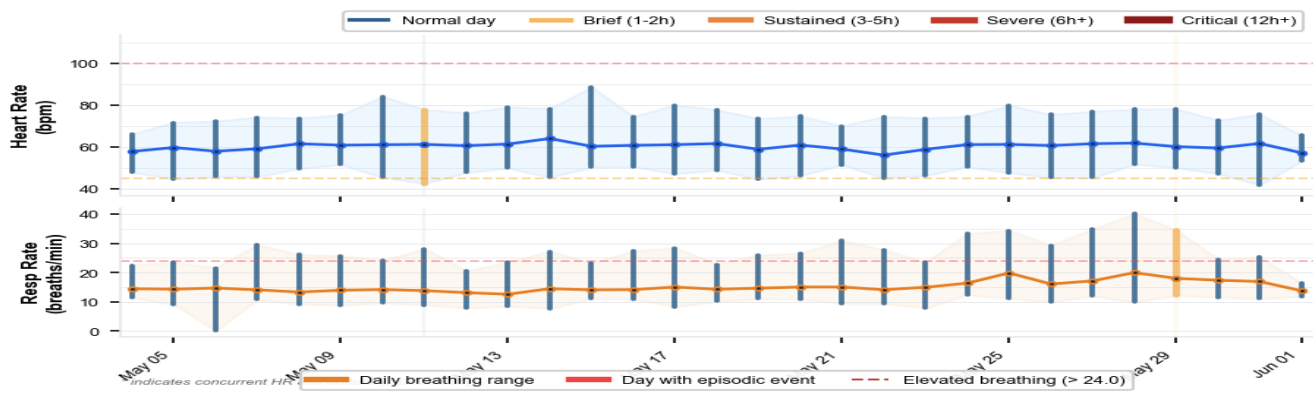
Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 11	11 AM to 12 PM	1h	1	Low Heart Rate	45	42	54	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
May 29	5 AM to 6 AM	1h	1	Elevated Breathing	25	14	35	Check SpO2 and lung sounds; assess for fluid overload or early infection

Daily recorded hours declined from 9.0 to 1.0 (89%) over the monitoring period.

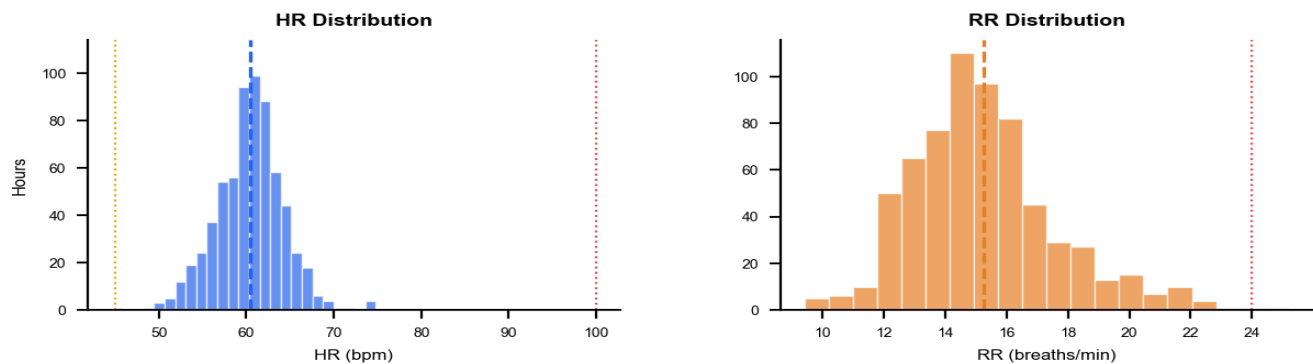
Suggested Clinical Review Actions

- Elevated Breathing (avg 25 brpm, peak 35 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Low Heart Rate (avg 45 bpm, min 42 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 2 unstable phases with 13 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.
- Daily recorded hours declined from 9.0 to 1.0 (89%) over the monitoring period.

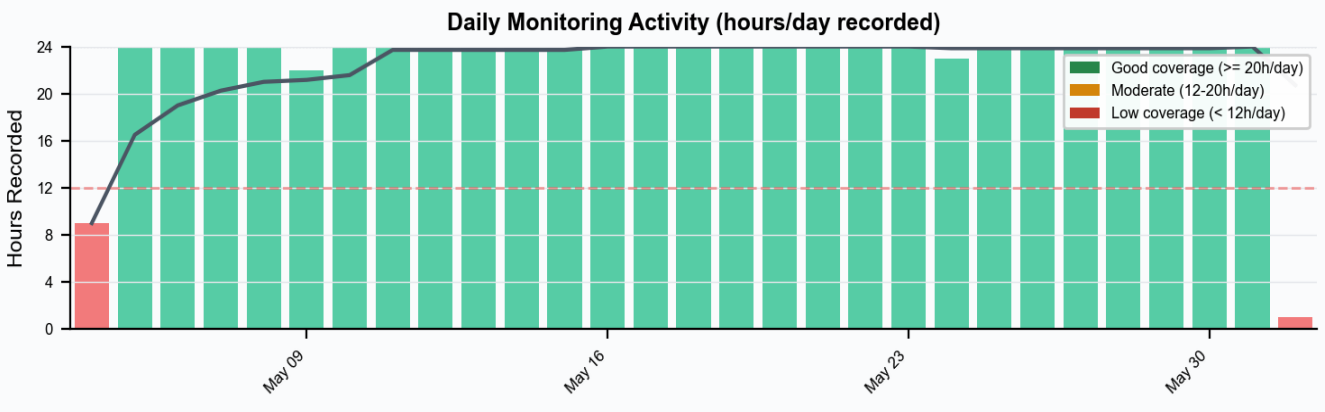


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

HR episode RR episode Recorded, no episode No data

Clinical Alerting Thresholds



Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.